جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : غاده محمد فتحي محمد
المحافظة : القاهره **الحي / المدينة :** خمسة عشر مايو **العنوان :** 15 مايو
رقم البطاقة : 28002080100181 **رقم التليفون :** 01060025163 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 08-02-1980 **المهنة :** حاصل علي بكالوريوس

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: غاده محمد فتحي محمد
National ID: 28002080100181



تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

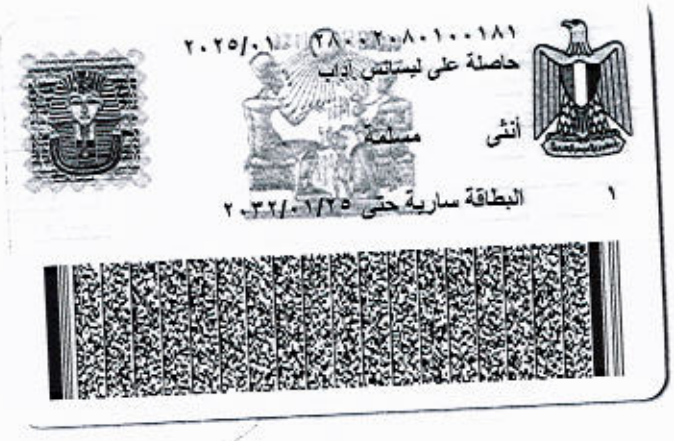
وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID · Front
[[DOCTYPE:ID_FRONT]]



National ID - Back
[[DOCTYPE:ID_BACK]]

Name: غادة محمد فتحي	
MR. : 9567	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 46-year-old individual with a known left breast invasive ductal carcinoma, HER2-positive and estrogen/progesterone receptor negative, initially staged cT4bN1M0. Neoadjuvant chemotherapy with the TCHP regimen was administered, followed by a left modified radical mastectomy in November 2023, which demonstrated a pathologic complete response. Adjuvant anti-HER2 therapy with Phesgo was completed through September 2024. The patient is currently on routine follow-up with no clinical evidence of recurrence. An abdominal and pelvic ultrasound has been requested, and routine laboratory studies are pending.

The patient is scheduled for follow-up imaging of a benign breast lesion, likely fibroadenoma, six months after the most recent assessment (BSM 4/2026). The left mastectomy site is clear, and the patient is in a postoperative bed pause as of May 2026. Evaluation for fatty liver disease with laboratory tests and a chest CT is planned, with the next visit scheduled in three months.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

Patient Name	: الأستاذة / غاده محمد فتحي محمد عبدالله	Request No.	: 13005947
Patient No.	: 28(0208100181	Request Date	: 25/11/2023
Path No.	: 4171/23	Verification Date	: 04/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نقدة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer (post NACT) .

Nature of the specimen: Left MRM

Gross Examination:
- Procedure: Left MRM

measured 21x16.5x4 cm with covering skin ellipse measured 17x13cm & attached axilla 9x6 cm .
- Tumor extent : Serialling showed grayish white areas up to 13x3x2 cm with ruberry grayish white cut section showing yellow foci, located 1 cm away from deep margin (least margin) .
- Also detected multiple circumscribed masses ranged from 0.7x0.7x0.5 cm to 3x2x2 cm with lobulated ruberry grayish white cut section showing focal calcification.
- Skin involvement: Nipple & areola were unremarkable grossly .
- Lymph nodes number and size: 13 L.Ns up to 2x1x0.5 cm .

Microscopic examination:
- Histological type: No residual viable tumor tissue detected
- Ductal carcinoma in situ: No
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/13)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic Complete response

Diagnosis:
Left MRM (post NACT) :
- No residual viable tumor tissue detected.
- No DCIs.

Verified by :
DR.Menna Nabil

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: Ghada Mohamed Fathy

Date: 29th April 2023

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: left breast invasive duct carcinoma grade II

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: c (heterogeneous dense breast parenchyma)

LEFT BREAST:

- **Low energy (Non contrast) images:**
  - Upper outer quadrant, anterior, middle & posterior thirds coarse heterogeneous calcification, fine linear branching pleomorphic microcalcifications
  - Architectural distortion mainly within middle third
  - Mild diffuse skin thickening
- **Recombined (Post contrast) images:**
  - Upper outer quadrant and central, anterior, middle and posterior thirds heterogeneous non-mass enhancing lesion with overall area of enhancement measuring 15 x 13 x 10.6 cm in CC, AP & transverse diameters respectively involving nipple/areola complex and skin
  - Axillary tail; irregular shaped spiculated margin mild mass enhancing lesion ?? lymph node.
- **Complementary US:**
  - **Upper outer quadrant & central 12-4 o'clock:** area of altered parenchymal echogenicity with internal calcifications reaching the nipple with skin thickening most prominent at 2 o'clock
  - **3 o'clock:** well-defined rounded marked hypoechoic solid lesion with internal calcifications measuring 2.5 x 1.5 cm in average diameter
  - ***More than three level I axillary lymph nodes; one of them showing irregular shape, ill-defined margin with focal cortical thickness 0.5 cm and eccentric hilum corresponding to the axillary tail mild enhancing lesions, features suggestive of extra-capsular rupture***

RIGHT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.
- Complementary ultrasound:
  - Heterogeneous dense breast parenchyma
  - 12 O'clock; well-defined oval shaped isodense solid lesion measuring 1.5 x 0.5 cm in average diameter, parallel orientation, no posterior acoustic features, probably benign lesion likely fibroadenoma

IMPRESSION:

LEFT BREAST: extensive pathological proved malignancy as described (BIRADS 6)

RIGHT BREAST: (BIRADS 3= probably benign lesion as described likely fibroadenoma)

Best regards,

Prof. Dr. Lamiaa Adel, MD

Female /Breast Imaging & Intervention Consultant

Cairo University

ت / ف: ٠٢٢٣٦٤٥٨١٣
Tel, Fax: 0223645813

١٠٥٣ ش كورنيش النيل ، قم الخليج ، القاهرة
1053 Corniche El Nil St. Kom Al Khaleeg , Cairo

Mammogram
Ultrasound

Patient Name	: الأستاذة / غاده محمد فتحي محمد عبدالله	Request No.	: 13003370
Patient No.	: 28002080100181	Request Date	: 28/03/2023
Path No.	: SR-403/23	Verification Date	: 06/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

CLINICAL DATA :
Left breast mass

NATURE OF SPECIMEN
Revision of tru cut biopsy &IHC slides

GROSS EXAMINATION
One block + 4 slides .

MICROSCOPIC EXAMINATION
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+3+3) Grade III
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
* IPT slides :
* ER: Negative nuclear reaction in tumor cells (Allred score 0/8)
* PR: Negative nuclear reaction in tumor cells (Allred score 0/8)
* HER2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
* KI67: Positive in 50% of tumor cells

DIAGNOSIS
Left breast mass ; slide revision of tru cut biopsy specimen :
Invasive duct carcinoma, Grade III .
* ER: Negative (score 0/8).
* PR: Negative (score 0/8).
* HER2: Positive (score 3+).
* KI67: Positive 50%.

Signature

Verified by :
Dr.Hebatallah Mostafa

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: Gada Muhammed	*Date of birth: 08/02/1980*
Arabic Name: غدة محمد	*Patient Gender: F*
Patient RIS ID: 327923 *Patient HIS ID: 123010264*	*Exam Date: 28/05/2023*
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (28/05/2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Dr/Omnia Talaat,MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Ghada Mohamed Fat'hy Mohamed Abd Allah	Date : 25/03/2023
Exam : C.T CHEST WITH CONTRAST	Age : 43 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. ***Chest:***

- Left breast multiple mass lesions harboring multiple calcific foci, the largest is seen measuring 2.2x 3.4 cm. diffuse overlying skin thickening is also noted.
- Few small left axillary level I and II lymph nodes the largest measures 0.9x 1.2 cm.
- No sizable pulmonary nodules, masses, cavitary lesions or consolidations
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is enlarged in size with homogenous low CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Ghada Mohamed Fat'hy Mohamed Abd Allah	Date : 25/03/2023
Exam : C.T CHEST WITH CONTRAST	Age : 43 Year(s)
Medical ID:	

- The scanned skeleton shows two small sclerotic osseous lesions are seen at sternal body and D9 vertebral body.
- Bilateral scaro-iliitis.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast malignant looking mass lesions associated with small ipsilateral axillary lymph nodes as described.***
- ***No definite pulmonary or hepatic metastatic deposits.***
- ***Two small sclerotic osseous lesions are seen at sternal body and D9 vertebral body, for bone scan correlation.***

Dr. Ghada H. Khalifa, MSc.
Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]